

Lecture 14 — Companion Figures

Hyperthyroidism & Thyroid Disorders — Prof Karim Meeran

roid follicular cell

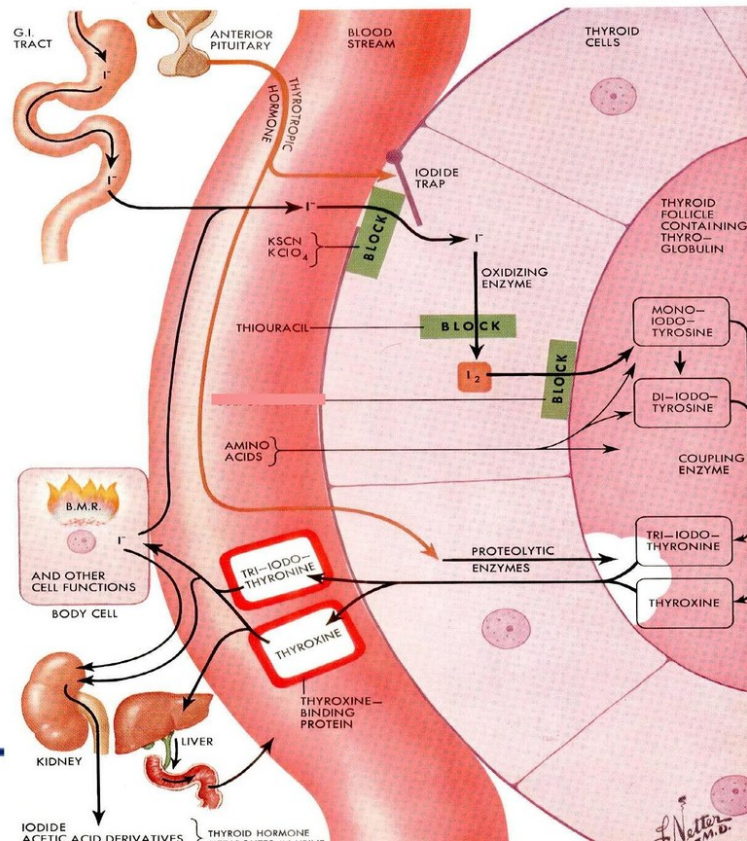


Figure L14.1 — Netter thyroid follicular cell: synthesis pathway and drug block sites

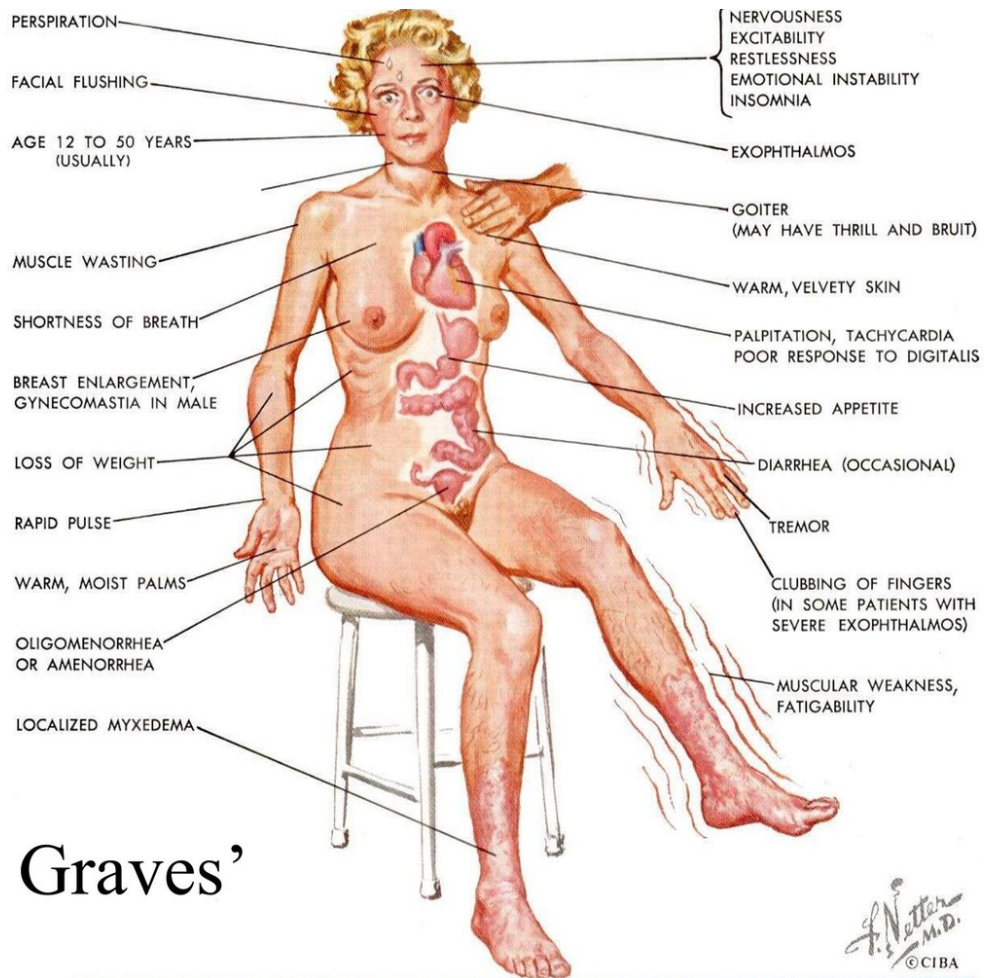


Figure L14.2 — Graves' clinical features (Netter whole-body figure)



Figure L14.3 — Exophthalmos: bilateral proptosis with sclera visible above iris

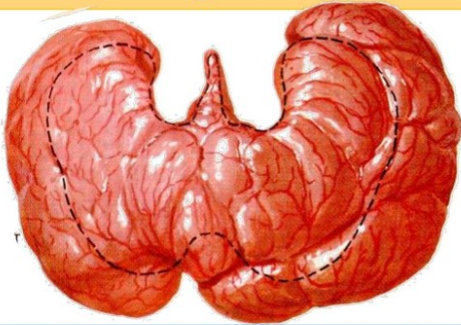


Figure L14.4 — Pretibial myxoedema: non-pitting, nodular, orange-peel skin of shins

Autoimmune disease

Antibodies bind to and stimulate TSH receptor in the thyroid

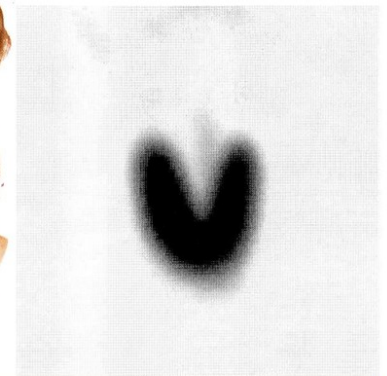
- Smooth **goitre**



Diffuse enlargement and engorgement of thyroid gland (broken line indicates normal size of gland)

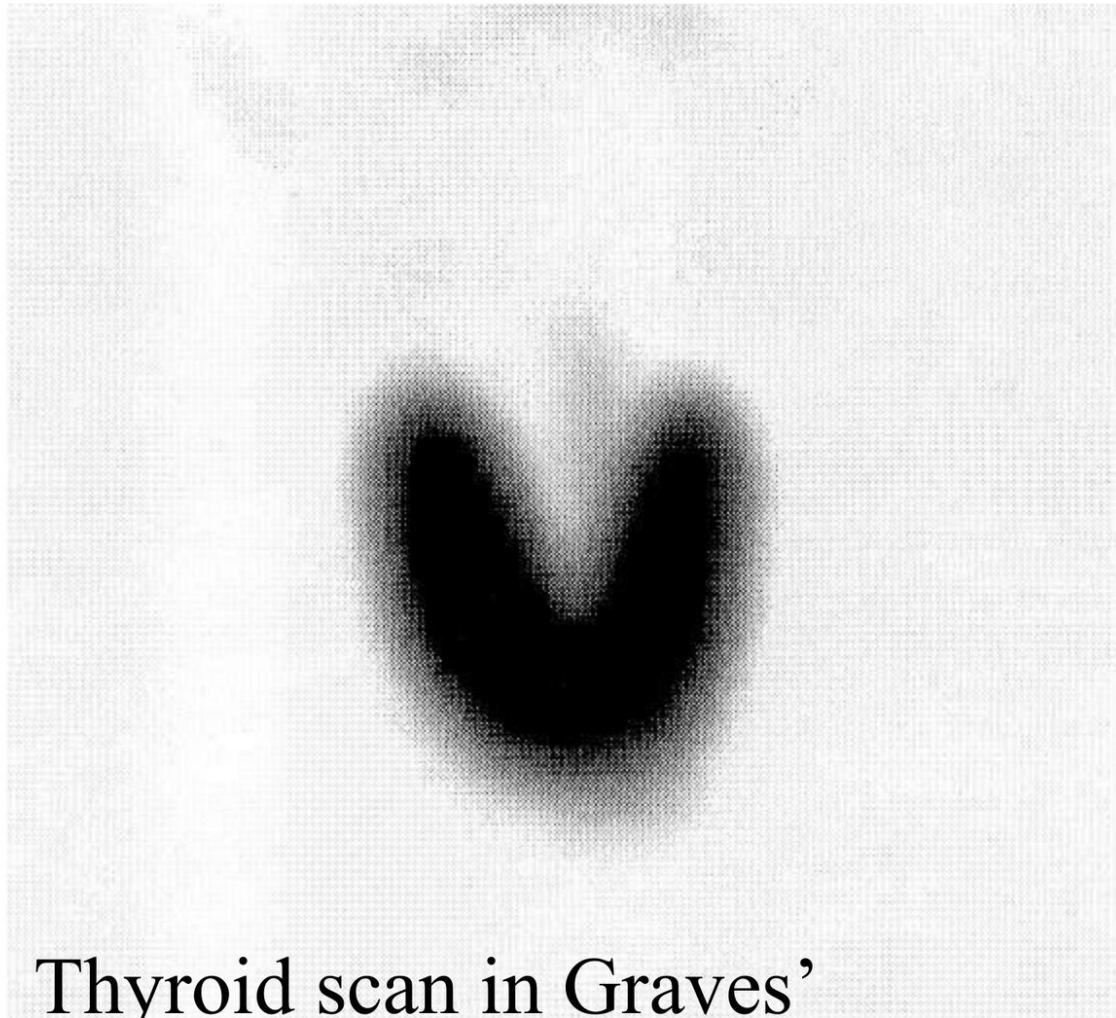
TSH-receptor antibody (TRAb) (against the TSH receptor)

- measured in the bloodstream, positive in Graves' disease
- first-line investigation to confirm Graves' disease



Diffuse goitre of moderate size and uniform radioiodine uptake

Figure L14.5 — Graves' disease composite: diffuse goitre + uniform radioiodine uptake



Thyroid scan in Graves'

Figure L14.6 — Graves' thyroid scan: symmetrical, uniform, intense uptake (butterfly)



Figure L14.7 — Toxic nodule: visible unilateral lump, asymmetric gland

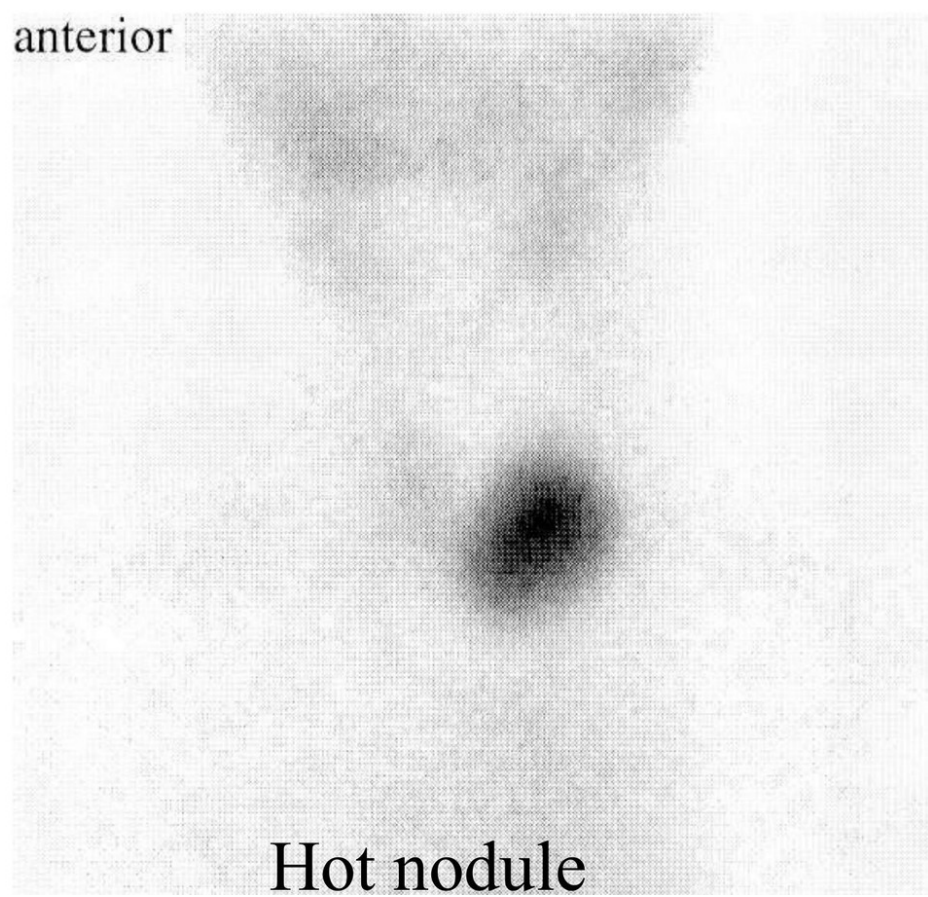


Figure L14.8 — Hot nodule scintigram: focal uptake, suppressed surrounding gland

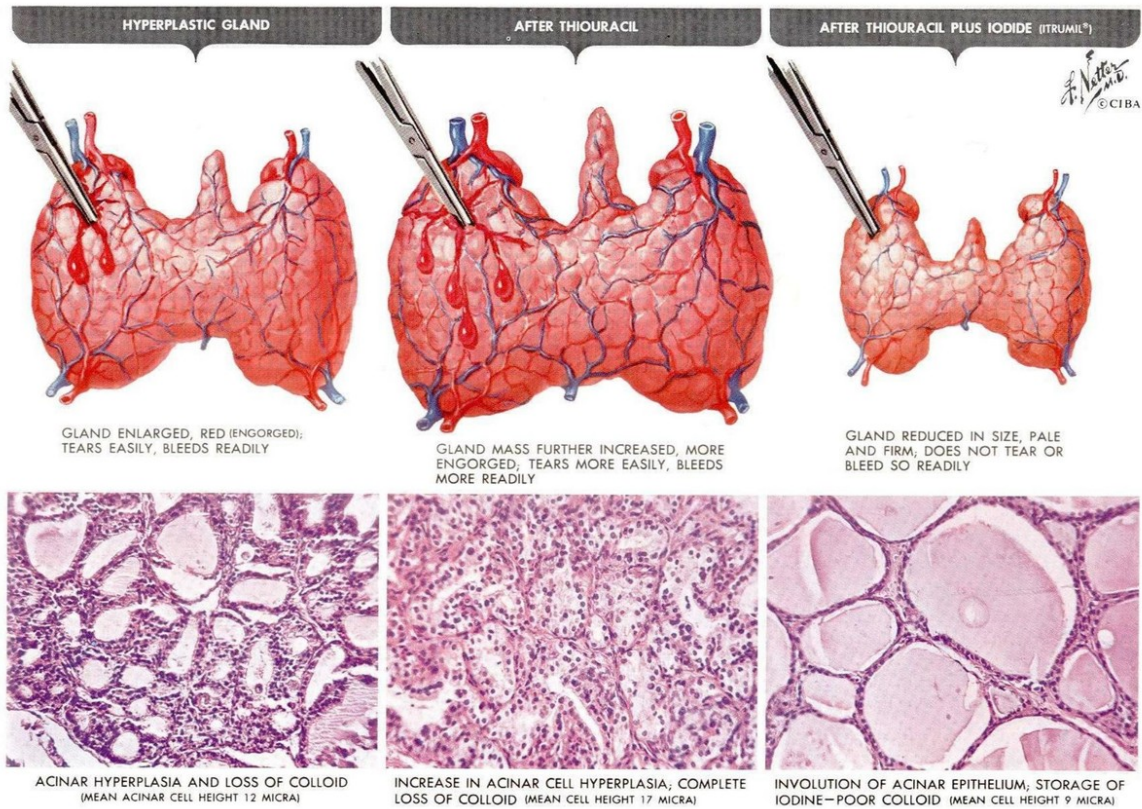
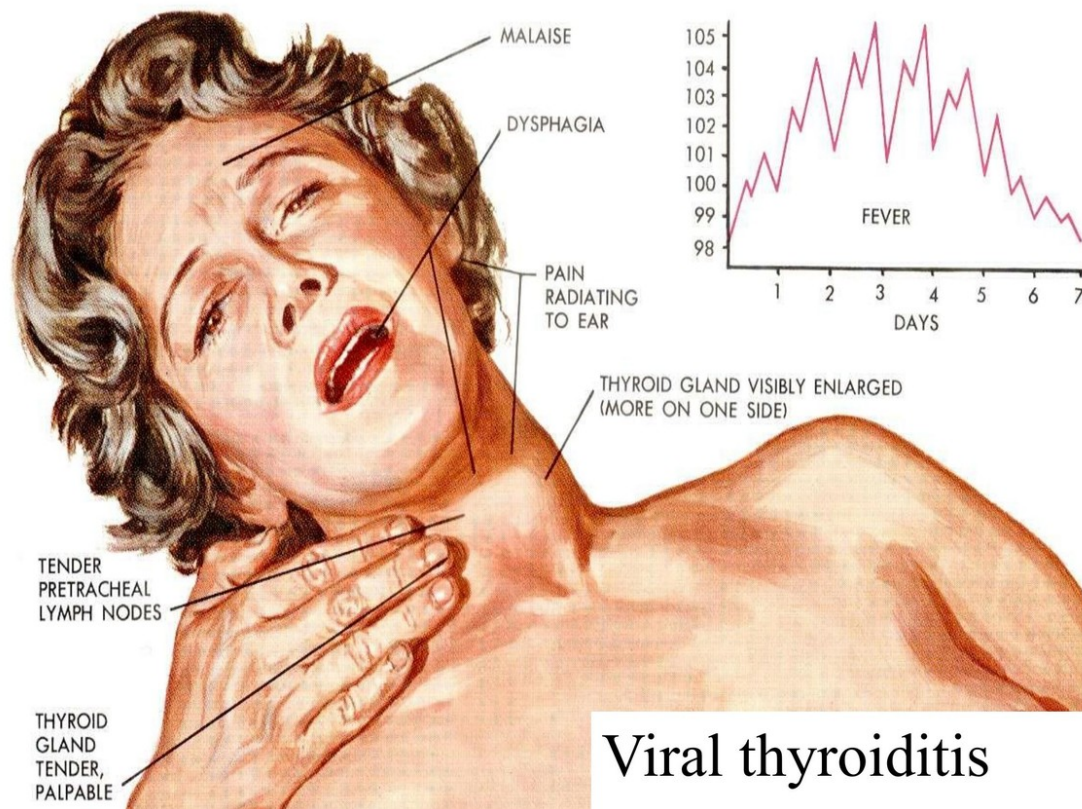


Figure L14.9 — Effect of thionamide ± iodide on gland vascularity and colloid storage



Viral thyroiditis

Figure L14.10 — de Quervain's viral thyroiditis: clinical picture with 7-day fever curve